

Leg Injury - Female (Child)

Algorithm 1304 | Child (0-17 years) | Gender at birth: Female

Definition

- Leg injury assessment, generalized from NHS.UK's published when-to-get-help guidance for limb injuries, parallel to the existing Arm Injury protocol.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] How did the injury happen?
2. [YES_NO] Can the person bear weight on the leg?
3. [DURATION] When did it happen?
4. [OPEN_TEXT] Record bleeding or open wound, foot colour, warmth, feeling, movement and pulse if trained, crush mechanism, head injury, age, pregnancy, anticoagulants, immune suppression, and safeguarding concern.
5. [YES_NO] If the patient is under 18, can they speak privately now, is the accompanying caregiver considered safe, and is there an approved safe callback method if contact is lost?
6. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Motor Vehicle Accident

Did this leg injury occur as part of a motor vehicle crash or a multi-system crash injury?

A motor vehicle crash or multi-system crash injury belongs in the Motor Vehicle Accident pathway.

1. Disposition level 100

If the patient is under 18, is harm occurring now, is there immediate danger from an accompanying or suspected caregiver, could the child be returned now to an unsafe person or place, or did contact end while an immediate threat remained unresolved?

Immediate danger, ongoing harm, an imminent unsafe return, or loss of contact during an unresolved immediate threat requires emergency action. Qatar 999 is the verified emergency route. Exact child-protection reporting, consent, documentation, and information-sharing requirements are local approval required.

2. Disposition level 78

If the patient is under 18 and there is no confirmed immediate danger, are they unable to speak privately, is caregiver safety uncertain, or is there another safeguarding concern requiring trained assessment?

An inability to obtain a private history, uncertainty about caregiver safety, or another safeguarding concern needs prompt trained assessment without automatically treating it as a confirmed emergency. The exact Qatar safeguarding pathway requires local approval.

3. Disposition level 100

Is there major deformity, exposed bone, uncontrolled bleeding, severe crush injury, rapidly increasing tense swelling or pain, or is the foot pale/blue/cold, pulseless, numb, weak, or unable to move?

NHS.UK limb-injury guidance lists visible deformity, protruding bone, and loss of feeling or circulation as emergency-assessment criteria.

3. Disposition level 70

Is the person unable to bear weight, or is there significant swelling and bruising?

Inability to bear weight is a recognized fracture indicator (the same logic underlying the Ottawa Ankle and Knee Rules already used in this system) warranting prompt assessment.

3. Disposition level 15

Is this a minor bump or strain with none of the features above?

A minor leg strain without deformity or weight-bearing difficulty is manageable at home.

Advice

Immediate child safeguarding danger

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

- Call Back If: harm occurring now or immediate danger; imminent return to an unsafe person or place; contact lost while an immediate threat remains unresolved.

Urgent private child safeguarding assessment

- Keep questions neutral and seek a private conversation only when it is safe. Do not disclose sensitive information to a caregiver whose safety is uncertain, do not confront or alert a suspected unsafe person, and do not promise confidentiality. Arrange prompt in-person assessment through the governance-approved Qatar child safeguarding pathway and record only an approved safe callback method. If immediate danger emerges or contact is lost during an unresolved immediate threat, use the emergency branch and call Qatar 999. Exact reporting, consent, documentation, and information-sharing rules are local approval required.

- Call Back If: private history cannot be obtained safely; caregiver safety remains uncertain; new immediate danger or loss of contact.

Emergency leg injury precautions

- Call Qatar 999 now. Do not realign or test the leg. Support it in the position found, cover exposed bone loosely, control bleeding around rather than on protruding bone, keep the patient warm, give nothing by mouth, and do not allow self-driving. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: foot becomes pale, blue, cold, pulseless, numb, or weak; rapidly increasing pain, swelling, bleeding, or collapse.

Prompt in-person leg assessment

- Use the Qatar governance-approved in-person service, avoid weight-bearing, support the limb, and use a wrapped cool pack. Do not massage or tightly bandage a significantly swollen or potentially fractured leg. Confirm medication for age, pregnancy, kidney disease, and bleeding risk.
- Call Back If: increasing pain or swelling, numbness, weakness, or foot colour/temperature change.

Care for a clearly minor leg injury

- Rest from aggravating activity and use a wrapped cool pack briefly. Do not tightly wrap the limb; confirm pain medicine with a pharmacist and reassess walking, swelling, colour, warmth, feeling, and movement.
- Call Back If: pain or swelling worsens, weight-bearing becomes difficult, or numbness, weakness, colour or temperature change develops.

First aid

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Qatar 999 now. Do not realign or test the leg. Support it in the position found, cover exposed bone loosely, control bleeding around rather than on protruding bone, keep the patient warm, give nothing by mouth, and do not allow self-driving. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Broken arm or wrist" (fracture red-flag criteria), <https://www.nhs.uk/conditions/broken-arm-or-wrist/> - generalized to leg injuries, parallel to the existing Arm Injury protocol (batch06)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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